



Safety and Best Practice Handbook



A Letter from CEO Dr. Ren Garcia and COO Marc Mercury

At Green Dog Dental & Wellness ("GREEN DOG" or "Clinic"), we know our most valuable and irreplaceable resource is the people that make up this beautiful family (the good kind of family). We recognize and respect the dedication our employees have put into perfecting their craft and without whom we would not exist.

For that very reason, our highest calling is the well-being and safety of our employees: physically, mentally, and emotionally.

This commitment is demonstrated through our extensive safety and best practice program. The program involves a continuous process of identifying, minimizing, and eliminating potential risks inherent in the operation of the company. Our efforts in safety include communicating the hazards to employees, educating and training employees, identifying and evaluating workplace hazards, investigating root causes of accidents/ injuries, hazard abatement or control, and implementing a disciplinary and recognition system that identifies willful employees and rewards employees with safe work ethics.

We can only achieve this high standard of well-being together. Without question, it is every employee's responsibility regardless of title or tenure. Accordingly, all employees are expected to take necessary actions to ensure that safe and healthy conditions and practices prevail.

No part of our business is more important than safety. All of us, through our leadership, commitment, and engagement must accept the challenge to work safely and actively to pursue injury and illness prevention. By being a good example, you will help us bring these policies to life and in the process create a culture that makes safety vital. Together, we can achieve our goal of a healthy, safe, and injury free workplace.

CEO: Dr. Ren Garcia

COO: Marc Mercury



WELCOME TO THE GREEN DOG FAMILY

As a GREEN DOG employee, we want to welcome you to our family of animal clinics. Working in an animal clinic is a rewarding experience. You will play an important part in helping pets and their owners. GREEN DOG is committed to maintaining a safe and healthy work environment for employees.

The GREEN DOG Safety Handbook is provided to each employee in their orientation kit, because we want all of our employees to understand how safety plays a role in the jobs they do every day. There are hazards in the veterinary workplace and we want to make sure you are aware of them so that you can do your job safely and efficiently.

This handbook addresses the basics of the Clinic Safety Program and the role of employees in the safety program. In addition, the handbook also provides required safety guidelines and procedures to help employees perform their jobs safely.

YOUR RESPONSIBILITY AS A GREEN DOG EMPLOYEE

The information provided in this handbook outlines the responsibility of employees, as well as the safety responsibility of clinic management. This information has been made available to you so that you may understand the efforts GREEN DOG is making to ensure your safety while working in our animal clinics and the goals of the entire clinic team. Once you understand your part in the process, as well as the responsibility of the leadership team, it will make it easier to comply with the program. You will have a better understanding of why they may ask you to perform certain tasks or perform tasks in a particular way.

We have provided our employees with tools, personal protective equipment, and information regarding safety in the workplace. It is your responsibility as a GREEN DOG employee to thoroughly review this information and adhere to GREEN DOG's safety guidelines and procedures. Please take the time to thoroughly read through this handbook.

Keep it on hand as a reference when you have questions about GREEN DOG's Clinic Safety Program and handling our patients safely.

GREEN DOG'S CLINIC SAFETY PROGRAM

GREEN DOG's Clinic Safety Program establishes an injury and illness prevention program and is made up of a number of safety systems, each addressing Occupational Safety and Health Administration (OSHA) regulations and federal law, and developed to ensure workplace safety for employees. To ensure a safe place of employment, the following safety systems, inherent in GREEN DOG's Clinic Safety Program, have been implemented:

- Injury and Illness Prevention Program (IIPP)
- Hazard Communication System (Haz-Com)
- Hazardous Drug Plan
- Emergency Action Plan (EAP)
- Waste Management Plan
- Radiation Protection
- Infection Control Plan
- GREEN DOG Clinic Safety Procedures

INJURY AND ILLNESS PREVENTION PROGRAM (IIPP)

An Injury and Illness Prevention Program (IIPP) is the written plan by a company or organization to prevent injuries and illness from occurring in the workplace. It provides the framework for a complete safety program for employees. The employer is charged with the responsibility to provide employees with safe tools and equipment, quality materials, established work procedures, and adequate rules and policies to maintain a safe place of employment.

GREEN DOG's Injury and Illness Prevention Policy

It is the policy of GREEN DOG Inc. to furnish a place of employment that is free from recognized hazards by proactively identifying, minimizing, and eliminating potential risks inherent in the operation of veterinary practice. Our objective is for every employee to be able to complete their work without injury to themselves, to others, clients, and patients. GREEN DOG's aggressive IIPP involves the Clinic management's commitment and responsibility to:

- Implement and maintain an effective Clinic Safety Program.
- Promote safety awareness and communicate workplace hazards to employees.
- Continuously identify, evaluate, and abate workplace hazards.
- Provide strong organizational policies and procedures.
- Provide injury and illness prevention training for employees, including
 - Providing personal protective equipment.
 - Accurate and timely report all occupational injuries and illnesses.
 - Perform accident and injury investigations to ensure proactive steps for prevention of future accidents.

EMPLOYEE'S ROLE AND RESPONSIBILITY IN SAFETY

It is understood that all Clinic team members will work with safety in mind and follow "good work practices". Employee's wholehearted acceptance of all aspects of the safety and health program, also includes the following role and responsibilities in safety:

- Adhere to all safety policies and procedures, including the use of required protective equipment and clothing.
- Accountable for knowledge and compliance of training material and safety procedures.
- Identify and report recognized workplace hazards and/ or deficiencies to the Clinic management team promptly.

- Report all injuries and/ or accidents immediately to a supervisor and seek medical attention when required.

Employees are expected to follow established procedures and to use common sense rules of safety. All GREEN DOG Clinic team members are held accountable and expected to cooperate fully to achieve these objectives.

SAFETY COMMUNICATION SYSTEM

A vital component to the safety program is the Safety Communication System. This system includes several approaches promoting safety awareness and communicating workplace hazards to employees. Supervisors will meet with their employees on an individual or group basis to discuss hazards associated with their jobs. The frequency of these meetings may depend on many factors, including employee turnover, purchase of new equipment, frequency of accidents, etc. Pre-planned employee safety meetings will be held as frequently as necessary to meet changing work conditions.

The following lists GREEN DOG Safety Communication efforts:

- Periodic Safety Meetings
- Safety Committee
- GREEN DOG Employee Safety Handbook
- GREEN DOG Safety Hazard Reporting Process
- GREEN DOG Working with Safety in Mind Posters
- New Hire Orientation Program
- Posters and Notices
- Other- Memos, Pamphlets, etc.

EMPLOYEE SAFETY TRAINING

Experience has shown that employee safety training is a valuable dissemination of safety information and the single most effective means of reducing injuries and illnesses in the workplace. We believe that the best way to prevent accidents and injuries from occurring in the workplace is to provide employees with the training they need to perform their jobs safely and efficiently. Clinic management is responsible for providing training, but employees are responsible to comply with the training provided to them.

GREEN DOG's Job Safety Orientation

As part of the New Hire Orientation, each employee is required to complete the New Hire Orientation, which outlines and documents job safety training. The safety portion of this training communicates and educates employees on GREEN DOG's Clinic Safety Program and Clinic safety policies and practices. Additional training may also be used and they vary widely with respect to instructional method, setting, subject matter, etc. The following is a list of training applicable to GREEN DOG's Safety Program, but not limited to:

- Lectures

- One-on-one sessions or on-the-job training, where a single or small number of employees receive personalized instruction from a supervisor.
- Simulations or demonstration
- Visual media (e.g., videos, PowerPoint presentations, etc.)
- Written instruction or training materials (e.g., brochures)
- Meetings with employees on specific training topics
- Training from an outside source (e.g., private consultants, vendor representatives, etc.)

HAZARD REPORTING

Employees have the right to raise a health and safety concern without fear of retaliation. Employees are also required to report unsafe conditions when noted. Communication with other employees, supervisors, safety coordinators, and Clinic managers is critical for the success of this program. Employees should report immediately any conditions they feel to be unsafe. This includes unrecognized hazards, unsafe acts, or infractions of safety rules. An employee will not be discriminated against for bringing safety issues to the attention of management. On the contrary, we will make every effort to reward meaningful safety suggestions and continued safe work practices.

Any employee or person (including visitors, clients, etc.) on the premises may report a hazard or submit comments, complaints, and suggestions on any safety and health issue. Reportable hazards include, but are not limited to, unsafe operations, procedures, practices, or conditions. The following are steps in hazard reporting:

1. To report a workplace hazard, you must complete a GREEN DOG Safety Hazard Report form. These forms should be made available and located in a central area at your Clinic. The form can also be found in GREEN DOG's Shared Google Docs. If you cannot get a hold of the form, contact your Clinic Manager.
2. You should specify the date, Clinic location, and the name of the person reporting the hazard for the purpose of verification or inquiries by Clinic managers. However, hazard reports may be submitted anonymously. Send reports to Jbabcock@greendogdental.com
3. Reference the event, unsafe practice or hazardous condition that affects the safety of personnel, and if any, the specific location in the Clinic.
4. The Clinic management team will review all Safety Hazard Reports and address the safety issue in a timely and effective manner.

ACCIDENT/INCIDENT REPORTING

Employees have a right to report an accident and/or injury without fear of retaliation or discrimination. It is also the responsibility of employees to report all accidents and injuries promptly to the supervisor on duty or to Clinic management.

When an incident or injury occurs:

1. Promptly notify the supervisor on duty or a member of the Clinic leadership team.

2. Actively participate in the accident investigation process.
3. The incident may be reported using GREEN DOG's Accident/Incident Form whether you plan to seek treatment or not.
4. In some cases understand that for your own safety you cannot return to work until an authorized return to work form is completed by your physician and turned in to your supervisor or Clinic Manager.
5. Follow work restriction guidelines as outlined by your physician if any.
6. Follow new operational procedures established to prevent similar injuries or illnesses from occurring in the future.
7. Ask for assistance when needed.

DISCIPLINARY ACTION FOR SAFETY VIOLATIONS

GREEN DOG has used the traditional disciplinary system, which is progressively corrective in nature, to deter employees from committing unsafe acts. A willful offender is an employee who purposefully commits a safety infraction after having received safety training specific to the task and/or operation of the employee's job description and assigned task, and is experienced in the job task being performed.

GREEN DOG recognizes and respects the fact that there is a difference between the employee who willfully, with knowledge, violates a safety practice and the employee who commits an error because they are inexperienced and/or have not received training or the training received was inadequate.

Corrective Discipline System

Managers and supervisors must use a corrective in nature approach to prevent accidents and/ or injuries. In seeking to implement management controls to correct the behavior of the employee, re-training is the most effective tool. The following is a procedure in re-training an employee who has been involved in an incident:

1. Review the applicable standard or safety procedure with the employee. Ensure that the employee's interpretation of the standard is the interpretation of the employer.
2. Review the deviation from the correct standard with the employee.
3. Review the application of the correct procedure with the employee.
4. Review the overall process and/or procedure with the employee emphasizing how the deviation can and must be avoided in order to avoid the incident.
5. Document training and file in employee's personnel file.

This corrective in nature disciplinary process is also an essential tool available for use in identifying the willfully, non-conforming employee. If a willful employee has been identified, please contact HR.

RECOGNITION FOR SAFETY

Employees who follow safe and healthy work practices will have this fact recognized and documented on their performance reviews.

HAZARD COMMUNICATION SYSTEM

Potentially hazardous chemicals can be found everywhere including work, home, and even during recreational activities. To ensure the safety of employees while performing their duties at the Clinic, GREEN DOG has implemented a Hazard Communication System(Haz-Com). Haz-Com is the written plan by a company or organization, required by OSHA, to effectively communicate workplace hazards to employees. Haz-Com is aligned with the United Nations' Globally Harmonized System of Classification and Labeling of Chemicals (GHS), which allows for international consistency in hazard classification. The plan identifies chemical hazards in the workplace and it requires the efforts of both the employer and employee to reduce exposures or eliminate them. The employer is charged with the responsibility to identify workplace hazards, identify methods to reduce hazards to employees, provide employees with tools and equipment to reduce exposure to hazards, provide quality materials, establish work procedures, and maintain a safe place of employment.

GREEN DOG's Haz-Com Policy

Our objective is for every employee to understand the hazards found in veterinary Clinics and to be able to complete their work without injury to themselves, to others, clients, and patients. GREEN DOG's Haz-Com System applies to operations in our Clinics where employees could be exposed to chemical hazards, both under normal working conditions and under emergency situations. Our Hazard Communication System also brings GREEN DOG Clinics into compliance with OSHA requirements for communicating hazards in the workplace.

We have committed our company resources to:

- Hazard Communication training by providing all employees with adequate training to acquaint them with the chemicals and other hazards that they may encounter in their job
- Assuring all employees are correctly using proper procedures when handling specific chemicals
- Monthly inspections to ensure that all containers are properly labeled
- Developing an Emergency Action Plan and a Fire Prevention Plan in case of spills or accidental releases
- Providing strong organizational policies and procedures
- Performing accident and injury investigations to identify hazards in the workplace and to ensure proactive steps for prevention of future accidents.
- Providing personal protective equipment (PPE)

Responsibility for GREEN DOG's Haz-Com System

To achieve the goal of working with safety in mind and total compliance with GREEN DOG's Clinic Safety Program, all aspects of the Haz-Com program must be learned, implemented, and followed.

It is the responsibility of Clinic management to provide employees with Haz-Com training and the tools or personal protective equipment to perform their jobs safely.

However, employees are responsible for the training material and practicing the safety procedures. Employees have access to information regarding workplace hazards and GREEN DOG's efforts to reduce or eliminate them. Employees are responsible for understanding the risks in the veterinary workplace. They are responsible for utilizing the resources GREEN DOG provides them. Haz-Com information is provided to GREEN DOG Clinic team members via the following sources:

- GREEN DOG Employee Safety Handbook
- Periodic Safety Topics
- Training Materials provided during meetings
- Safety Training Videos/Pamphlets/Posters/Handouts

Employees should report immediately conditions they feel to be unsafe, including, unrecognized hazards or infractions of safety rules, so that they can be corrected as soon as possible.

Communication with other employees, supervisors, safety coordinators and Clinic managers is critical for the success of GREEN DOG's Clinic Safety Program.

COMPONENTS OF HAZARD COMMUNICATION IN THE CLINIC

Pictograms are also located on SDSs to warn the user of the hazards associated with a particular chemical product.

Corrosion  • Skin corrosion/ burns • Eye Damage • Corrosive to metals	Exclamation Mark  • Irritant (skin and eye) • Skin sensitizer • Acute toxicity(harmful) • Narcotic effects • Respiratory tract irritant • Hazardous to ozone (non-mandatory)	Flame  • Flammables • Pyrophorics • Self- heating • Emits flammable gas • Self-reactives • Organic peroxides
Gas Cylinder  • Gases under pressure	Flame Over Circle  • Oxidizers	Exploding Bomb  • Explosives • Self-reactives • Organic peroxides
Skull and Crossbones  • Acute toxicity (fatal or toxic)	Health Hazard  • Carcinogen • Mutagenicity • Reproductive toxicity • Respiratory sensitizer • Target organ toxicity • Aspiration toxicity	Environment*  • Aquatic toxicity*

HAZARDOUS DRUG PLAN

Some GREEN DOG Clinics may handle hazardous drugs, such as chemotherapy drugs. Hazardous drugs are drugs which display one or more of the following characteristics: carcinogenicity, teratogenicity or fertility impairment, serious organ dysfunction or other toxic manifestation. OSHA identifies antineoplastic cytotoxic medications, anesthetic agents, antiviral agents, and others, as hazardous drugs. These hazardous medications are capable of causing serious effects including cancer, organ toxicity, fertility problems, genetic damage, and birth defects if protective measures are not in place.

The risk of exposure to hazardous drugs is greatest during drug preparation and administration with the primary routes being inhalation of dusts, direct contact or dermal absorption, and ingestion through contact with contaminated food or cigarettes. Two other routes of exposure include handling of disposed items that have come in contact with hazardous drug agents (syringes, catheters, gloves, etc.) and contact with excrement from patients treated with chemotherapy. In addition, both oral and injectable medications have the potential to be hazardous.

To protect employees and minimize occupational exposures to the risks of adverse health effects when handling these drugs, Clinics have a written Hazardous Drug Safety Plan. The plan requires employees are appropriately trained, ensures employees comply with GREEN DOG safety procedures, and are wearing proper PPE, proper disposal of waste, etc.

EMERGENCY ACTION PLAN

OSHA requires a written Emergency Action Plan (EAP) at each facility to inform employees of what to do in case of an emergency. A floor plan should be posted in each room with the information listed below.

Emergency Action Plan

It is the Clinic management's responsibility to create an emergency action plan and train all employees on proper emergency procedures.

All employees should be trained on what to do in the case of an emergency including:

- What to do in case of certain types of disasters or threats more prevalent to the area, such as an earthquake, hurricane, tornado, fire, workplace violence or medical emergency
- Where to locate the EAP floor plan
- Where the nearest emergency exit(s) and fire extinguishers are located
- Where to find safety equipment
- How to handle safety equipment
- What are the emergency phone procedures
- What are the proper evacuation procedures
- Where to seek refuge or meet after evacuating the area

Follow your Emergency Action Plan

It is the responsibility of every employee in the Clinic to follow the Emergency Action Plan as they have been trained. Follow these basic steps in an emergency situation.

- Keep safety in mind when faced with an emergency. It is important, for the safety of everyone, that everyone stays composed.
- Evacuate the building if necessary and meet outside in the parking lot, or other identified relocation area, for a head count.
- Walk briskly. Do not run.
- Alert everyone else in the facility about the emergency.
- Call the appropriate agency to handle the situation.
- It is important that all employees know the locations of, and how to safely turn off main gas and water lines as well as handling other safety equipment such as fire extinguishers.
- Use the emergency phone list next to the phone to call the appropriate agency such as police, fire rescue, or ambulance.

Evacuation Plan

An evacuation plan should be pre-determined and practiced routinely. Each employee should know:

- The alarm or sound that indicates an evacuation
- The location to be used for a safe refuge or relocation site.

- A head count will be conducted. (Employees cannot leave until they have been accounted for.)
- All employees may return to the building once the “All Clear” is announced.

If a member of the staff is missing, the emergency response personnel will be notified immediately.

WASTE MANAGEMENT PLAN

Most waste generated in veterinary Clinics can be handled normally by tossing it into a trash bin or draining it into the sewer system. However, a significant portion of the waste falls into classes known as medical waste or hazardous waste, which require special handling to protect not only employees, but the general public and the environment at large. There is the potential risk to personnel involved in the disposal of some of these wastes, and to the public, if it is not managed correctly; and the potential for pollution of the environment or visual offense if wastes are not disposed of properly.

GREEN DOG’s Waste Management Policy

To protect employees and persons from injuries and the environment due to these types of hazards, Clinic management must maintain standardized procedures for the handling, storage, hauling, and disposal of medical and hazardous waste. It is the responsibility of Clinic management to ensure proper waste management at each facility for the prevention of injuries and illnesses.

Medical Waste

Medical waste is waste generated at healthcare facilities including animal Clinics. GREEN DOG contracts with approved waste haulers to dispose of these varied wastes properly. Types of medical waste include:

- Biohazardous (or infectious waste) and sharps waste
- Pathology waste (animal tissues, organs, and parts that are not infectious)
- Pharmaceutical waste (including controlled substances)
- Chemotherapy or chemo trace waste (e.g., contaminated gloves and gowns, used syringes, empty chemo vials, and tubing used during chemotherapy administration)

Medical waste is waste, either solid or liquid, generated or produced as a result of the diagnosis, treatment, or immunization of human beings or animals and may present a threat of infection to humans. The main type of medical waste produced in veterinary Clinics is biohazardous waste, which includes sharps waste.

Biohazardous Waste

Biohazardous waste or infectious waste is the following:

- All cultures, including negative growths.
- Vaccine vials which contained Brucellosis vaccine or Contagious Ecthyma vaccine.

- Animal parts, tissues, fluids, or carcasses suspected of being contaminated with infectious agents known to be contagious to humans.
 - Waste or containers with recognizable blood from animals known to be infected with diseases that are highly communicable to humans.
 - Waste containing excretions, exudates, or secretions from isolated animals known to be infected with diseases that are highly communicable to humans.
- Tissues fixed in formaldehyde or other fixatives.

Sharps Waste

Sharps waste is defined as any device having acute rigid corners, edges, or protuberances capable of cutting or piercing, including but not limited to, all of the following:

- Hypodermic needles, hypodermic needles with syringes, blades, needles with attached tubing, syringes contaminated with biohazardous waste, acupuncture needles, and root canal files.
- Broken glass items, such as Pasteur pipettes and blood vials contaminated with biohazardous waste.

All employees handling, disposing, and packaging medical waste must wear protective clothing. Proper personal protective equipment (P.P.E.) may include gloves (or double gloves), face shields, masks, eye protection, gowns and aprons to protect against exposure to potentially infectious materials or hazardous materials.

All medical waste must be identified and segregated from the other waste streams at its point of origin (or the room or area at which medical waste is generated) into its proper container to ensure proper treatment and disposal of waste.

- All biohazardous waste (non-sharp waste) must be disposed of directly into a biohazard bag.
- All sharps must be discarded into leak-proof, puncture resistant containers. A sharps container is a rigid puncture-resistant container that, when sealed, is leak resistant and cannot be reopened without great difficulty. Available from the Clinic's hazardous waste hauler.
- Chemotherapeutic agents must be segregated for storage, and, when placed in a secondary container, that container shall be labeled with the words

“Chemotherapy Waste” or “CHEMO” on the lid and on the sides, so as to be visible from any lateral direction, to ensure treatment of the biohazardous waste. Sharps waste exposed to chemotherapeutic agents must be placed in sharps containers labeled with “Chemotherapy Waste” or “CHEMO” and segregated to ensure treatment of the sharps waste.

- Pharmaceutical and controlled substances waste are disposed of through a reverse distributor, such as Med-Flats or Guaranteed Returns.

When filled, all medical waste must be properly sealed.

- Biohazard bags must be tied and placed in rigid containers for storage, handling, or transport. Biohazard bags must be impervious to moisture, of sufficient strength to preclude rupture under normal usage, and certified by the bag manufacturer to pass the impact resistance test. Do not use trash or laundry for handling medical waste.
- Biohazard bags must be placed for storage, handling, and transport in rigid containers with tight-fitting lids, which may be disposable, reusable, or recyclable. Containers shall be leak resistant, have tight-fitting covers, and be kept clean and in good repair.
- Sharp containers must be approved, leak resistant, have tight-fitting covers, and are clean and in good condition. All full sharp containers that are ready for disposal should be taped closed or tightly lid. Full sharps containers may be placed in a biohazard bag.
- Chemo waste must be approved, leak resistant, and have tight-fitting cover containers.

In case of emergency, when surfaces are contaminated with spilled or leaked biomedical waste they must be cleaned with a solution of industrial strength detergent to remove visible soil and must be disinfected. The following are required procedures:

- Exposure to hot water of at least 180 Fahrenheit for a minimum of fifteen (15) seconds.
- Exposure to chemical sanitizer by rinsing with, or immersion in, one of the following for a minimum of three (3) minutes:
 - Hypochlorite solution (500 ppm available chlorine)
 - Phenolic solution (500 ppm active agent)
 - Iodoform solution (100 ppm available iodine)
 - Quaternary ammonium solution (400 ppm active agent)

Personnel performing disinfection procedures shall be equipped with the appropriate personal protective equipment for the situation, but at a minimum shall wear chemical eye protection and latex gloves. Protective clothing, shoes, and a face shield may be required for large quantities of biohazardous materials. Liquid waste created by these chemical disinfecting operations may be disposed of into a sewage system.

All clients who generate biohazardous waste in their home must be informed verbally and in writing of the recommended method for handling and packaging of biohazardous waste. GREEN DOG recommended method is to purchase additional small sharps containers, where clients can ship sharps waste for disposal, at an added cost to the client. Clinic employees should refuse the receipt of sharps waste from a client, especially if the sharps waste is not stored in an approved sharps container or the sharps container is leaking and not in good condition.

It is extremely important to follow proper protocol when handling biohazardous waste and sharps waste, as these items can injure the employee, and if contaminated, can also cause infection or transmit disease. The following are general safety requirements:

- Do not mix biomedical waste with regular trash unless the whole container will be treated as biomedical waste.
- Do not dispose of hazardous waste as medical waste. Hazardous waste requires special handling.

- Do not remove bagged biohazardous waste from the biohazard bag until the waste is treated.
- Wash and decontaminate reusable, rigid containers each time they are emptied, unless protected by disposable bags or liners.
- Use only the medical waste treatment method allowed for your Clinic's classification.
- Syringes and needles should be disposed of in the sharps container.
- Do not separate needles from syringes.
- Do not re-cap needles.
- Wrap the used scalpel blade in its foil wrapper when removing from the instrument.
- Always use a hemostat when removing scalpel blades or needles.
- Do not point an exposed needle at anyone.
- Never shake a sharps container.

HAZARDOUS WASTE

Hazardous waste (normally in the form of chemical waste) also has properties that make it potentially dangerous or harmful to human health or the environment. The EPA has developed a list of hazardous wastes that are ignitable, corrosive, reactive or toxic under the Resource Conservation and Recovery Act (RCRA). Examples of hazardous waste (or chemical waste) in GREEN DOG Clinics are bulk chemo drugs, used carbolime, formaldehyde, DipQuick stains, radiographic developer and fixer, etc.

Classifying this type of waste can be done by reviewing the physical and chemical properties section of a product's SDS and comparing it to the EPA list. Once classified as hazardous wastes, it must be disposed of into a container labeled "Hazardous," normally provided by an approved hazardous waste hauler and should not be disposed of down the drain. If the hazardous chemical is liquid, dispose of the liquid in secondary containment and place it in the hazardous bin. Disposal of this waste is done by an approved hazardous waste hauler in your local area.

Waste Anesthetic Gases

Inhalation anesthetics (such as halothane, isoflurane, and nitrous oxide) are used almost universally in veterinary practices. However, these waste anesthetic gases and vapors may pose a potential harm to employees if not controlled. Some potential effects of exposure to waste anesthetic gases (WAGs) are nausea, dizziness, headaches, fatigue, and irritability, as well as sterility, miscarriages, birth defects, cancer, and liver and kidney disease. Check with the SDS for specifics.

To help control this hazard and keep employees safe, every GREEN DOG animal Clinic that conducts surgeries and/or dentals are equipped with a proper scavenger system, such as an active scavenging system, passive exhaust, and absorption (or charcoal absorption).

However, to ensure the system is in good working condition, scavenging systems must be monitored by the use of monitor badges, which is done at least twice a year. In addition, anesthetic machines are calibrated and serviced on a regular basis to reduce potential leaks.

Although you may have the best active scavenger system on the market, it will not do you any good if the system is not turned on. Therefore establishing safety work practices are just as imperative. In order to reduce the amount of waste anesthetic gas in the Clinic environment, the following procedures are required in the use of anesthesia gases and machines:

- Ensure the waste gas scavenging system is connected and working before using the machine.
- All anesthesia machines are expected to have a functioning scavenger system.
- Check that lines, hoses, scavenging equipment and absorbers are properly connected and checked for leaks at the beginning of each procedure.
- Always use a mask that fits snugly over the patient's nose and mouth without excessive escape of gas around the rim. When masking a patient, select the smallest mask size available and still provide a proper fit. This minimizes anesthetic gas that can escape from around the mask.
- Make sure the mask is free of cracks or defects that may allow gas to leak into the air.
- Remember to turn on the oxygen, place the mask on the patient, and then turn on the flow of gas. Reverse the order to remove the mask.
- Make sure to use appropriate flow rates for the size of the patients.
- When not in use, the vaporizers and oxygen should always remain off.
- Use properly fitted endotracheal tubes and/or masks. Loose-fitting tubes are a common source of exposure especially for employees who are working close to the head of anesthetized animals.
- Inflate endotracheal cuff after placement. **DO NOT OVER INFLATE ENDOTRACHEAL TUBES.** Over inflation can cause life threatening tracheal tears.

If you are NOT familiar with endotracheal cuffs, ask for assistance.

- After shutting off the vaporizer, continue to administer oxygen a few minutes longer before disconnecting the patient.
- This flushes anesthetic gas out of the patient's lungs and into the scavenging system.
- Check the soda lime crystals regularly and change when needed (every 12 hours of use).
- Be sure to record when soda-lime has been changed in the Clinic maintenance binder and directly on the machine.
- Turn off anesthetic gas and oxygen before filling.

RADIATION PROTECTION

Most veterinary Clinics perform radiographic procedures on a daily basis. Therefore, it is important that all employees are familiar with radiation safety and strictly adhere to the radiology safety procedures. X-rays are invisible masses of electrons set in motion through an X-ray tube. Although invisible, their effect can be significant, dangerous and cumulative if appropriate safety measures are not in place. If you are unaware of the dangers of X-rays, please check with your supervisor who will be happy to provide you with additional information.

GREEN DOG'S RADIATION PROTECTION POLICY

In order to protect employees from the exposure of radiation, GREEN DOG will provide employees with the necessary tools, personal protective equipment (PPE), information on the hazard, and training to prevent injuries. It is the responsibility of Clinic management to provide the training and the tools. However, employees are responsible to comply with safety guidelines and procedures, including wearing mandatory PPE.

“GREEN DOG New Hire- Previous/Concurrent Radiation Exposure History” form, which is provided to them in the New Hire packets. This form must be returned to the supervisor. In addition, employees should provide their supervisor with their “termination radiation exposure report,” which employees can request from their previous employer. Employees are also responsible for informing their supervisor of any concurrent employment where they are being monitored for occupational radiation exposure and were issued a dosimeter.

Guidelines of Radiation Safety

When involved in the use of radiographic equipment, the following are general guidelines for radiation safety:

- Areas or rooms that contain permanently installed X-ray machines must be posted with a sign reading “Caution X-ray” with an international symbol.
- No person under 18 years of age shall be allowed to be an operator, restrain animals for x-ray exposure or be present in the x-ray room during an exposure.
- Often, the state regulatory agency will inspect machines to ensure they are working properly, but only the operator can use them safely. Therefore, before the use of an x-ray machine, operators should be familiar with the location of all the controls and the function of every individual knob or button.
- The operator must be trained on the physical or health hazards associated with radiation. They must know and understand and follow GREEN DOG's Radiation Protection Program for proper operation of equipment.
- Personnel must rotate through radiology, no one person is allowed to take ALL of the radiographs.
- Only the operator shall be in the x-ray room or area while exposures are being taken unless assistance is required.
- Operators shall be instructed to, and shall be held responsible for, clearing the room when an exposure is to be taken.
- Whenever possible, exposures shall be taken with no persons in the x-ray room.
- If the operator and/or assistants must restrain, hold or support an animal during an exposure, these persons MUST wear personal protective equipment (PPE) which include:
 - Lead Aprons
 - Thyroid Shields
 - Lead gloves on their hands (THE HAND MUST BE INSIDE THE GLOVE.)
 - Paddles for shielding hands are not acceptable substitutes and should not be in the Clinic inventory.

- o Personal protective equipment must be in proper repair and should be tested at regular intervals.
- o The use of positioning aids, tie down restraint and chemical restraint is to be encouraged through the cooperation of the veterinarian.
- o It is often best for both the patient and the handler that chemical restraint is used on nervous or fractious animals.
- o GREEN DOG policy prohibits the use of excessive force in the restraint of a patient when other means of restraint are available. The goal is to reduce employee exposure to radiation by minimizing the duration of time that the employee is in the x-ray room during exposure.
- o The use of exposure time, tube distance, shielding, and conscientious collimation shall be used to keep exposure to radiation a minimum.
- o X-ray units must be calibrated yearly. This is not just the service of the x-ray processing unit. Specifically, this is the calibration of the machine that produces the x-ray radiation.
- o Record calibration in the Clinic safety maintenance binder.
- o Always use the lowest possible x-ray exposure setting to achieve a diagnostic radiograph.
- o Always attempt to position the patient properly the first time - retakes are not only wasteful but exposes employees to unnecessary radiation.
- o Films and x-ray cassettes will not be hand held during exposure.
- o Members of the public are not allowed in the radiation area during exposure.
- o If the operator and/or assistants must remain in the x-ray room during an exposure, every effort must be made to be as far away from the primary x-ray beam as possible.
- o The PRIMARY X-RAY BEAM shall strike no part of the body, even with protective lead gloves in place.
- o Annual training in radiation safety is required for all operators of diagnostic X-ray equipment.

Animal Patient Holding

1. No individual is employed or regularly used to hold animal patients during radiation exposures.
2. Veterinary radiographers do not hold animal patients except very infrequently and then only in cases in which no other method of restraint is available.
3. Use devices to assist in positioning and restraining the anesthetized or sedated patient (such as cassette holders, sandbags).
4. If manual restraint must be used, require the individual to hold the animal patient at arm's length with the body positioned as far away from the animal patient as possible. (The head and body may not be bent over the animal patient.)

Portable machines, like those used in large animal and mobile practices, can be particularly dangerous because of their multi-purpose abilities. These machines can be "aimed" in any direction, and because of their limited "power" they must utilize longer exposure times to produce diagnostic images.

- o When using a portable machine, always make sure no one is in the path of the primary beam (even at a distance).
- o Never hold a cassette with your hands, gloved or not; always utilize a holding pole.
- o Remember to wear a lead apron and gloves.

ALARA (As Low As Reasonably Achievable)

To aid in the prevention of injuries or illnesses due to radiation, Clinic management are required to limit employee exposure to radiation. Technicians should be aware that one of the best protection devices against radiation absorption is avoiding unnecessary exposure to x-rays. The level of exposure should be As Low As Reasonably Achievable (ALARA). This can be done in three ways:

1. Time: Decrease the time one is exposed to x-rays.

- o Avoid having to retake film. Stay organized, measure correctly, and do not rush. Make sure you have a correct technique chart.
- o Use positioning aids and tie downs whenever possible. Tie down all sedated pets. Take x-rays from behind a lead shield whenever possible.
- o Consider chemical restraint on fractious pets.

2. Distance: Increasing the distance from the source

- o With the use of ropes, sandbags and other positioning devices, the technician is able to restrain animals, to take various positioned x-rays while increasing the distance from the source of the x-ray.
- o When using manual restraint, lean away from the x-ray table.
- o Never allow any part of the body to be under the primary beam.

3. Shielding: Use of protective clothing containing lead.

- o Every GREEN DOG Clinic is equipped with lead protective gear such as lead gloves, lead aprons and thyroid shields. Each employee is also expected to wear a radiation-monitoring device.
- o The use of radiology safety protective equipment is NOT OPTIONAL. Failure to use safety equipment is grounds for strict disciplinary action and/or termination.
- o All pieces of lead radiology protective equipment must be x-rayed for compromise of lead twice per year as a minimum. They should be x-rayed more often if they receive heavy use. If equipment is found to be faulty, immediately contact your supervisor to discuss replacement.

Record inspections of radiation safety equipment in the Clinic safety maintenance binder.

A dosimeter badge is a radiation-monitoring device that measures the amounts of radiation exposure employees receive in a given time-period. This badge must always be worn during radiographic procedures, not as protection, but as a measurement of any scatter radiation an employee may receive during the procedure.

- o Every employee who is involved with radiation on the job must have and utilize an individual dosimeter.
- o Dosimeters must be worn every time an employee takes an x-ray.
- o The placement of dosimeters depends on state regulations. Contact your supervisor or manager for specifics. (For example, in California the whole body badge is worn on the outside of the lead apron on the collar.)
- o At the end of an x-ray monitoring period, the dosimeters badges are sent back for analysis. Reports will be posted with radiation exposure limits.
- o Dosimeters should never leave the Clinic with an employee. Never wear dosimeters outside because exposure to sunlight will result in false readings.
- o Dosimeters should never be stored in the radiology room. Specifically, they should never be stored on the apron itself! Please ensure that the dosimeters are stored away from radiation scatter or direct sunlight in a convenient area where employees can gain access to them.

It is imperative that all employees have access to their radiation exposure levels. These reports, required by law, are designed as a warning system to alert the employee and the supervisor or manager if an employee's exposure to radiation increases. GREEN DOG's Clinic Safety Program requires that radiation reports be posted as soon as they are received. Employees are required to remember their participant numbers to identify their reading on the reports; personal information is omitted from the report for the protection of employees.

The radiation reports can be confusing to those who do not know how to read them.

Listed below is an explanation of how to read the report.

- o The explanation and the reports should be posted together in a central location accessible to all employees.
- o The radiation report explanation is also a part of employee safety training.
- o The importance of wearing dosimeters should be reviewed at safety meetings. Information on how to read the dosimeter report is also located on the back of the radiation report.
- o It is the Technical Supervisor's responsibility to monitor the reports to be sure that employees are not being over-exposed to radiation. The x-ray monitoring report compiles information regarding:
 - o Current monitoring period radiation exposure levels
 - o Year-to-date radiation exposure levels
 - o Lifetime radiation exposure levels.

In addition to the Radiation Dosimetry Reports, it is required by the U.S. Nuclear

Regulatory Commission (NRC) and other state regulations to provide every employee, exposed to radiation, an annual radiation summary report or NRC Form 5. If you have not received your NRC Form 5 report, contact the Clinic management team.

Annual Radiation Exposure Limits

- o DEEP (DDE): Whole body, blood forming organs, gonads 5,000 mrem/yr
- o EYE: Lens of eye 15,000 mrem/yr
- o SHALLOW: Extremities and skin 50,000 mrem/yr
- o Fetal 500 mrem/yr
- o General Public 100 mrem/yr

MINIMAL DOSE EQUIVALENT REPORTED:

Dose equivalent below the minimum measurable quantity for the current monitoring period are recorded as “M”. The minimal reporting levels vary according to the dosimeter type and radiation quality as follows:

- o X-ray, Gamma 1 mrem
- o Beta 10 mrem
- o Neutron 20 mrem
- o Fetal 1 mrem
- o Rings 30 mrem

Based on USNEC Regulation, Title 10, Part 20, Code of Regulations and adopted by many states. Certain states and other regulatory agencies may adhere to different limits.

Employees who are pregnant or potentially pregnant should be protected to minimize exposure of radiation. All women in this practice have a right of privacy regarding pregnancy. Those employees who voluntarily declare that they are pregnant will be asked to sign a formal statement to that effect and will receive instruction by Clinic management in procedures to minimize exposure. Dosimeters may be given to a pregnant employee to measure fetal exposure for the duration of the pregnancy.

The following constitutes as an Emergency:

- o Loss or theft of radiation device
- o High or potentially high radiation exposure to an individual or to a member of the public.
For example:
 - o Greater than 500 mrem in a short period of time to any occupationally exposed individual;
 - o Great than 5000 mrem in a short period of time to the extremity of any occupational exposed individual; or,
 - o Greater than 10 mrem to any member of the public.
- o Deceptive or potentially deceptive exposure of a dosimeter.
- o Any personnel injuries, which may involve radiation exposure.

In the event of an emergency, employees must contact the Clinic management team immediately.

GREEN DOG CLINIC SAFETY PROCEDURES

The safety of our employees, clients, and patients is very important. Our goal is to operate our Clinics with minimum risk to employees, patients and clients. We have dedicated resources to provide our employees with the information and tools they need to succeed in their jobs as well as guidelines and systems of accountability. We are asking that every employee understand this and apply the tools we have provided.

GENERAL SAFETY PROCEDURES

- o Report every accident.
- o Report any condition or practice to your supervisor, safety coordinator or GREEN DOG' Safety Personnel, which you consider to be unsafe or unhealthy, preferably, before an accident occurs.
- o Notify your supervisor whenever you are taking prescription medication that may interfere with your daily work assignments.
- o Operate only the equipment on which you have been instructed and trained.
- o When operating the equipment, follow only company-approved operating instructions.
- o Smoking is not permitted inside the premises.
- o The use of alcohol and drugs during working hours will be cause for dismissal.
- o Horseplay and practical jokes are prohibited.
- o Safety equipment is provided. It is the employees' responsibility to maintain it and wear it when required.
- o Employees are required to present a "return to work" slip when returning from time off work as a result of a job related injury.
- o It is the responsibility of each employee to know and follow all accident prevention instructions and safety precautions for their job.
- o Obey all warning signs.
- o Always wear appropriate footwear for the job.
- o Walk, never run.
- o Safety devices on equipment should never be circumvented.

Housekeeping

All employees are responsible for maintaining a clean work area. Good housekeeping helps to eliminate unsafe conditions. When the workplace is clean and orderly and housekeeping becomes a standard part of operations, less time and effort will be necessary to keep the facility clean. Operations can be performed more efficiently and quality can be improved. An effective housekeeping program is a planned offensive against accidents/injuries. It is not characterized by an occasional grand cleanup. Good housekeeping will result in a decline in accidents and promote greater worker satisfaction.

- Use daily, weekly and monthly checklists to ensure housekeeping tasks are regularly completed. Daily and weekly tasks should be periodically reviewed to accommodate a changing Clinic environment.

- Use the periodic maintenance checklist to keep track of receipts and invoices for the maintenance of equipment.
- Trash, fur, loose syringe caps and other materials should be picked up off the floor in a timely manner and discarded in appropriate receptacles.
- Discard of waste appropriately (e.g., sharps waste into sharps container, biohazard waste into biohazard container, etc.); do not let waste containers overflow
- Clean up spills immediately or if they are too large, use “Caution Wet Floor” signs and clean up as soon as possible.
- All tools and equipment must be kept in good and safe working condition. Damaged tools or equipment should be removed from service and discarded or tagged “defective” when first detected.

If using a ladder, be sure to use safe ladder practices when performing the task

- Read and follow all labels/ markings on the ladder.
- Inspect the ladder and take time to properly position the ladder as needed. Tag out any ladders that are damaged.
- Always maintain a 3-point (two hands and a foot, or two feet and a hand) contact on the ladder when climbing. Keep your body near the middle of the step and always face the ladder.
- Be sure that it is sturdy and firmly set at a safe angle.
- Avoid stretching to reach to the side.
- Keep the area around the ladder clear from obstructions.
- Never use the top step or rung of a ladder as a step.
- When working with new chemicals, always refer to the SDS to find out about recommended PPE, spill response and first aid recommendations.
- Store chemicals in a secured manner
- Store housekeeping materials and equipment safely and neatly. Avoid clutter that might cause accidents.
- Keep walkways free of clutter to prevent tripping related injuries.
- Never block doors, hallways, fire exits, or utility appliances.

Electrical Equipment

Electric hazards aren't like other hazards in that they can't be seen unless obvious signs are present such as frayed wires or sparking. Special attention must be paid around electrical equipment and when in doubt, always assume that equipment is energized:

- Disconnect the power source before servicing or repairing any electrical equipment. In emergency situations, shut down the power remotely if you can.
- Risk of electric shock is greater in areas that are wet or damp. Be sure to install GFCI (Ground Fault Circuit Interrupters), grounded 3-prong outlets.
- Never wear rings or metal watch bands when working with electrical equipment.
- Extension cords are for temporary use. Install permanent wiring when use is no longer temporary.

- Inspect portable cord-and-plug connected equipment, extension cords, power strips, cables, electrical fittings, etc. for damage or wear before each use. Repair or replace damaged equipment immediately. Post an “out of order- do not use” sign when necessary.
- When replacing lamps and bulbs, verify that the replacement matches fixture requirements.
- Treat all electrical devices as if they are alive or energized.
- Always tape extension cords to walls or floors when necessary. Nails and staples can damage extension cords causing fire and shock hazards. Keep power cords out of aisles and walkways.
- Know where the panel and circuit breakers are located in case of an emergency.
- Do not block access to panels and circuit breakers or fuse boxes. Keep a 3-foot clearance.
- Use all electrical equipment in accordance with the manufacturer’s instructions.
- Daisy-chaining is dangerous! Do not connect a power strip or an extension cord to another power strip or extension cord.
- Electrical equipment with high voltage, such as the UPS should be plugged in directly to a properly wired GFCI.
- Metal or aluminum ladders must not be used while working on or near electrical equipment.

Lifting

Working in a veterinary Clinic is a physical job. Daily, employees are transporting and restraining patients as well as lifting other objects. If not careful, employees can cause themselves severe harm if they do not follow procedures for lifting and transporting patients. Unfortunately, many do not realize the risks and proceed without caution, taking for granted that they will not hurt themselves “just this one time.”

Please follow the guidelines below when lifting heavy objects:

1. Never hesitate to request assistance.
2. Do not lift or transport an object or patient that is over 40 lbs. without assistance.
3. If assistance is not immediately available – do not proceed! Wait until help arrives.
4. Always try to utilize a helping device such as: gurney, stretcher, dolly, hand truck, or cart.
5. Before attempting to lift a heavy object or animal, be sure the surrounding area and the path you will be following are clear of objects and free of slip hazards.
6. Get a firm footing. Place your feet a shoulder’s width apart.
7. Get as close as possible to the object before attempting to lift.
8. Do not reach over objects to lift.
9. Grip your load firmly.
10. Bend at the knees, not your waist. Keep back straight.
11. Stand up and lift the object with the action of the leg muscles. Let your leg muscles do the work.
12. Lift slowly in a fluid motion.
13. Keep the load close to your body. For greater strength and stability, lift and carry the object near your waist.

14. Do not lift boxes above your shoulder.
15. Pivot, do not twist if you need to turn. Move your feet when changing directions.

MEDICAL EMERGENCIES

Medical emergencies include cardiac arrest, injuries to the head, neck or back, open wounds, and conditions requiring professional medical assistance.

Life Threatening Conditions

- Call 911 for professional assistance.
- Describe the situation to the dispatcher in detail. Stay on the line until told to hang up.

Non-Life Threatening Medical Emergency

- Make the injured worker comfortable, but do not move them. Keep the person warm, dry, and calm.
- Refer the injured worker, if he/she can walk, to a GREEN DOG approved physician.
- If the injured worker cannot walk, call 911 for assistance.
- Do NOT attempt to render first aid treatment on a person. (However, if an employee is designated as a First Responder during emergency situations and is trained to perform CPR/first aid, follow the Clinic's Bloodborne Pathogen Program, including protocols on wearing proper P.P.E. for protection.)
- Designated First Responders have the right to (but are not required to receive) a Hepatitis B Vaccination.
- If an employee has been bitten or scratched by an animal, he/she may SELFRENDER first aid treatment only by:
 - Washing the area thoroughly with antibacterial soap.
 - Applying an antiseptic barrier.
 - Applying pressure to stop bleeding.
 - Applying a padded bandage to keep the wound clean.
 - Applying ice to reduce swelling.
 - Keeping the bandage clean.
- Seek treatment by a physician immediately if deemed necessary.

Emergency Eyewash Stations

There may be rare situations in which you may need to use the eyewash station. Every GREEN DOG Clinic is equipped with at least one eyewash station. Eyewash stations must be located within approximately 55 feet (or 10 seconds walking time) without obstruction to areas where there are caustic chemicals present. These areas may include: radiology, treatment, kennel, grooming/bathing, and lab. Employees should know the location of the nearest eyewash station and how to use it.

Eyewash stations are activated with one motion and are easy to operate. Most equipment is designed for hands-free operation with either paddles, levers, or a pull lever. To turn off, simply

reverse the action required to turn it on. Eyewash stations are checked monthly to ensure proper operation as part of the Clinic Safety Inspection

Checklist. Ask your supervisor or Safety Coordinator to show you how to use the equipment.

To use emergency eye wash:

- Immediately flush eyes for at least 15 minutes.
- Keep the eyes open and rotate the eyeballs in all directions to remove contamination from around the eyes. An injured person may need help holding the eyelids open.
- Call the Poison Control Center at (800) 222-1222 for advice, then seek medical attention immediately.
- Have someone review the Safety Data Sheet and send a copy to the doctor if requested.
- Report injury or exposure to your supervisor promptly.

PATIENT SAFETY

Patient safety and employee safety go hand in hand. One of the top priorities for the entire staff is the safety and welfare of the pets in our care. Many procedures to keep patients safe and comfortable will also keep our employees safe. Sometimes employees get injured when they are trying to correct an unsafe situation for a pet.

Here is a real life example: An 80 lbs. golden retriever was waking up from anesthesia on a treatment table; he was not being closely supervised. As he became more active in the recovery process, he started to slip off the table, a technician (120lbs) noticed that he was going to crash to the floor, and tried to scoop the dog up as it was halfway off the table by herself as there was no one available to help. In the process of doing this, because the dog weighed 2/3 her own weight, she injured her back. This could have been prevented if proper patient safety guidelines were followed. As you read through the information listed below, realize that each of the safety points listed, when ignored, have been causes of injury to both patients and employees.

Patient Welfare

- If a pet is barking, whining or scratching at the cage or kennel, it is the immediate responsibility of the staff to make sure there is no physical reason for the animal's stress.
- Restlessness and barking may indicate the patient's need for water or an opportunity to relieve itself.
- If ignored, the pet may be subjected to undue stress and the possibility of soiling itself from salivation, vomiting, or eliminating in the cage.

Kennel, Treatment and Grooming

- Make certain all compartment doors are securely latched.
- Help pets out of upper cages, do not allow them to leap out of cages or leap into your arms.
- Never allow a pet to leap off of a high surface, as it can easily injure itself or an employee.
- Striking an animal may be cause for termination or disciplinary action.

- When bathing pets, be sure to constantly monitor the water temperature.
- When using a cage dryer, check the pet every few minutes to make sure it is not suffering from heat stress and never set a dryer on high.
- Never leave an animal unattended in a tub, exam room, exam table or grooming table (even if the pet is tied up).
- Never leave a tied up animal unattended. It can easily choke itself.
- Keep pets under control at all times.
- Require that clients have their animals confined or on leashes while in the waiting area and on leashes while walking their pet. A loose pet is in danger of getting away and becoming lost. A loose pet is also in danger of causing fights and harming other animals or employees.

Anesthetic Procedures

- Patients waking up from anesthetic are in danger of injuring themselves or employees. Closely monitor their recovery. Watch for the swallowing reflex and remove the endotracheal tube as soon as possible.
- A pet that is intubated must be monitored constantly, as they can easily aspirate the tube. Take caution when removing the tube. Be sure to keep fingers out of the mouth.
- If a patient is being dried and recovering from anesthetic, be sure to place them at the rear of the cage so that they do not suffer from heat stress. These patients must be monitored more closely.
- Use caution when transporting large dogs that are incapacitated or under anesthesia.
- Not asking for assistance puts both the employee AND the patient at risk of injury.

Clinicization

- During business hours there should always be at least one employee on the premises who can effectively monitor patients, assist in a crisis, and who is trained to administer CPR to a patient in need.
- Patients on fluids require consistent monitoring to ensure they receive the prescribed volume. If a patient receives an excessive amount of fluids there can be fatal results.

Getting Patients out of the Clinic in the Case of an Emergency

In an emergency, it is imperative that human life be considered the first priority. Only after it is deemed that human life is not at risk, shall patients be evacuated. When evacuating patients, if possible, follow the procedures listed below.

In an emergency, the order of priority is:

1. The safety of humans,
2. The safety of animals, and
3. The preservation of property.

At no time will the safety of people be jeopardized unnecessarily for any reason.

- It is imperative that all patients who are admitted to the Clinic are properly labeled with name tags and cage cards ROUTINELY for purposes such as emergencies.
- All people (employees, clients, visitors, volunteers, etc.) must exit the building immediately unless engaged directly in surgery, anesthesia, or intensive care activities at the time of the alarm.
- Patients housed in the kennels, runs or wards should not be transported unless employees are in the area at the time the alarm sounds and evacuation of animals does not impede the timely evacuation of humans from the building. These individuals may lead or carry patients when the individual is exiting the building but may not remain in the building during the alarm, go to another part of the building, or return to the building after evacuation until allowed to do so.
- No one who exits the building may re-enter unless directed to do so by emergency personnel.
- Clinic personnel engaged in surgery, anesthesia induction, or intensive care units may temporarily remain in the building when the evacuation alarm however must leave the building immediately if instructed to do so regardless of whether or not animals are ready to be transported. Clinic personnel in the essential areas must continually evaluate the conditions in their immediate work area and the surrounding hallways and workspaces. If a hazardous condition occurs in their work area (such as fire, smoke, or chemical fumes), they must immediately exit from that area, even if an evacuation order has not been given.
- If all else fails, release pets to the outside. While this is not optimum practice, it is far better to allow the animal freedom of escape rather than to be stuck inside. We recognize that it may be difficult to round up all of the pets that have been released outside. Still, this is a better alternative and most pet owners will appreciate efforts to save their pets.
- If an immediate evacuation is ordered, all personnel must exit the building immediately. Emergency personnel will enter the building to check for people and may remove animals when possible to do so. Every effort will be made to rescue animals when reasonable to do so and when the safety of people is not jeopardized.
- Designate several areas to be used for housing evacuated animals (e.g. carriers, cars, etc.). Consider coordinating with the nearest GREEN DOG Clinic if possible.
- Leashes, lead ropes, pillowcases, portable carriers, and/ or large rolling cages should be made available for immediate action.
- Keep a log of animals in the Clinic and update it as animals come in and out. The log should include the animal's name, owner's name and contact information.

ANIMAL HANDLING AND RESTRAINT

It is very important to follow safety procedures when handling animals. Veterinary Clinic employees come in contact with animals daily. Most injuries occur when a patient is being restrained for treatment. Properly following animal handling safety rules will often prevent an

employee from being bitten, scratched or injured. Please pay special attention to the following guidelines:

- Personal protective equipment (PPE) should be worn when warranted.
- Never attempt to lift a heavy animal alone. When handling animals, always observe the proper lifting techniques, bend your knees, and keep the animal close to your body. Grasp the animal firmly. Keep your chin tucked in. Lift with your legs. Never twist your body while carrying an animal, always pivot.
- Recognize that animals may react differently to new situations. Therefore, never let your guard down.
- Always approach animals in a calm, slow, non-threatening manner.
- Keep your face out of the reach of the patient's muzzle at all times. Never kiss a pet.
- Once you have a pet properly restrained, keep a firm hold on the patient's head and neck. Do not let go unless there is danger of choking the patient, or the veterinarian treating the patient indicates to do so.
- Animals can act unpredictably when recovering from anesthesia. Be especially alert during this time.

Adequate Training

- The worst accidents and injuries occur at a veterinary Clinic when employees are trying to restrain pets without adequate help and/or training. Never attempt to restrain any patient if you do not have adequate training.
- If you are faced with this situation, enlist the help of more qualified personnel and be sure to notify a supervisor that you require more training in restraint techniques.
- Failure to do so can be cause for disciplinary action. You are putting yourself and the patient at risk as well as others.
- Do not attempt to handle any pets until you are completely familiar with proper handling and restraint techniques.

Restraint Devices

- Restraint devices have been provided for your use. Do not hesitate to use them. The simple act of muzzling a questionable pet has saved many employees from injury.
- Check equipment to be sure it is in operable condition and replace restraint equipment as needed. There is nothing worse than getting a muzzle on an aggressive dog only to find out the closing device is broken.

Chemical Restraint for non-aggressive patients

- If the patient is struggling excessively, it may be necessary to utilize chemical restraint. Chemical restraint or tranquilization reduces the need for forceful handling and therefore

makes the entire treatment experience more pleasant for the Veterinarian, employee and patient.

- If an employee believes chemical restraint is indicated, please do not hesitate to discuss it (privately) with the patient's veterinarian who will make the final decision.
- Employees should feel confident that chemical restraint is properly used at their facility and should discuss concerns further with their supervisor if they have questions about this issue.

Animal bites and scratches

If you are bitten while working with a pet:

- Wash the area thoroughly and apply an antiseptic barrier such as betadine.
- Apply a bandage and keep the bandages fresh and clean. Dog bites can produce a lot of bleeding; apply pressure with thick padding or gauze.
- Notify the supervisor and submit an accident report immediately.
- Employees are advised to seek treatment by a physician immediately.

Handling and Managing Aggressive Patients

Remember that most aggressive behavior is fear based. Put yourself in the patient's fur. Try to imagine what it would be like to go to a doctor's office and have uncomfortable procedures done to you with no explanation or ability to understand that it will help you in the long run. Rough handling can often drive an already fearful patient to the point of aggression. Keep in mind that safety for our patients and employees are of utmost importance.

- Hostile and aggressive animals must be handled appropriately. If an employee is not familiar with proper techniques to handle these types of patients, they are instructed to seek assistance. Failure to do so could result in serious injury to both patient and employee.

Proper Identification of Aggressive Patients

- Proper identification of potentially aggressive animals can help an unsuspecting employee avoid a bite or scratch, especially when the patient seems totally manageable during treatment or admission to the Clinic.
- If a patient exhibits aggressive behavior for the first time, be sure to indicate it in the patient chart. Provide details about the incident and if applicable, include what set the patient off. For example; a patient might be "cage shy". This means that the patient behaves well out-side of the cage but when putting them into and out of a cage they react in an aggressive manner.
- If an animal is a "Caution", or "Escape Artist", it should be written in red and with large enough letters on the cage card so that it can be easily seen on the cage.
- Record information about the patient's disposition, personality and if it has bitten anyone in the patient chart. Use a red pen and large letters in the patient's chart to warn staff members of the potential dangers when handling this patient.

- Be sure to check the chart and cage card for warnings BEFORE working with ANY patient. Proper warning can greatly reduce the incident of bites and scratches.
- Always approach a fearful pet with caution and use a calm tone of voice.
- Never make the assumption that a patient will not harm you when frightened and always be sure to keep your face out of the “bite zone”.

Chemical Restraint for aggressive patients

- If a patient exhibits excessive aggression, do not hesitate to discuss chemical restraint and/or sedation with the patient’s doctor. This may benefit the patient and the employee. Many accidents could have been avoided (and valuable time saved) if a more proactive approach to chemical restraint had been taken.
- Avoid using excessive force in restraining an animal whenever possible.
- We encourage our doctors and staff to utilize chemical restraint.

Handling Cats

- All cats should be transported to the Clinic and from the lobby to cages in a carrier or pillowcase.
- If at all possible, allow cats to come out of the carrier box or pillowcase on their own.
- Most cats will react favorably if you do not make them feel as if you are “doing something” to them. Move slowly without using force, petting or visiting with them often works well during simple procedures.
- When picking up a cat, place one hand under its chest, restraining front legs with fingers. Use the second hand to support the rear end.
- Restrain the cat’s head by using a hand underneath the neck.
- Never allow a cat’s mouth close to your fingers or face.
- Grasping a cat by the scruff should only be done if the cat has expressed fearful tendencies.
- Never forget that cats are very athletic animals. They possess more than their fair share of armament and can quickly become very unsafe to handle.
- Several varieties of restraint techniques may be used, such as boxes, stocks or “cat bags”.
- Placing a cat on a slippery surface such as a metal exam table can be helpful as the cat will have trouble keeping its footing and will not be able to spare a paw for scratching.

Handling and Managing Aggressive Cats

Cats can greatly differ in temperament. Temperament can change drastically from behavior at home to behavior at the Clinic. Most often cats react due to fear. They will struggle to avoid being confined, and when pain is inflicted can go into a fit of wild rage with the appearance similar to the “Tasmanian Devil “ cartoon. The following lists an overview of information when dealing with frightened cats:

- Fearful cats will try to appear as small as possible. They flatten ears and tail and will attempt to move away from the frightening source, usually by crouching and moving to the back of the cage.

- Cats will most often growl, hiss or spit before attacking. Take this warning seriously.
- During heavy restraint, cats will be highly motivated to escape. These cats will go into the “Tasmanian Devil” stage of flying teeth and claws. Sometimes it is best to let go, and allow the cat to calm down and re-group.
- Never let go of the patient without warning any person who might be assisting you first.
- Cats are much more likely to go under, over or behind furnishings. Releasing heavy restraint while still maintaining control of the situation is important to prevent cats from going where they will be hard to retrieve.
- Use techniques that may work well on aggressive cats. Muzzles, cat bags, acrylic restraint box, heavy towels, and/or large rolls of paper towels for biting diversion.
- Use the roll of paper towels between the cat’s front legs, and underneath the chin. This allows the cat to release aggression by biting and scratching at the roll of paper towels, therefore distracting it from the handler.

Handling Dogs

Always take care when approaching a strange dog. Move slowly and speak calmly. Dogs seldom bite or attack without warning. Be sensitive to the dog as an individual. Use the following guidelines when approaching and working with a dog:

- Care should be taken to cause as little pain by manipulation and examination as is necessary to effectively evaluate the patient.
- Key breeds to always approach with caution: Chow Chows, German Shepherds, Dalmatians, Rottweilers, Akitas, Sled Dog breeds, Cocker Spaniels, Dachshunds, and a variety of Terrier breeds including the Pitbull.

Understanding how human behavior affects the dog’s perception

- Never surprise a dog. When approaching a dog, make sure it can hear you and see you. This is especially important with older dogs.
- Standing over, leaning over, hugging or petting a strange dog may be interpreted as a dominant act and could elicit a bite.
- Make brief eye contact. Do not “stare down” a dog. Let a dog sniff you and carry out its own preliminary investigation of you.
- If a person makes a quick movement toward a dog before it has sniffed or completed sniffing, it may be misconstrued as a threat.
- Never put your hand on the top of a dog’s head when greeting it. This is considered dominant and/or aggressive behavior by the receiving dog. Stand in a relaxed manner, you are less threatening and less vulnerable than when in a crouched position.
- Move your hand slowly and deliberately along the dog’s body. Canine body language
- Always be aware of changes in the dog’s body language. Watch out for a “frozen” posture, standing on tiptoes, curled lip, raised hair. Never wait for a growl of displeasure.
- Key body language postures of a friendly dog are tail wagging and fluid body motion (wiggling).

Dog Fights

- Stop the behavior at the very instance you see it about to escalate. Call for back-up. Never try to break up a dog fight alone.
- Take care when leading a dog past other pets. Do not allow any pet to come in contact with another animal.
- Never put your hand in the way when trying to break up a dog fight as it is an easy way to get bitten.
- Use a mop or broom handle to help separate the dogs or use a barrier to split them up, such as a large cardboard or plywood without putting your hands in harm's way. Splash a bucket of water over the dogs or throw a thick blanket over them in hope to disorient them.
- In extreme circumstances, consider noise devices (e.g., air horns and whistles) to distract the dogs' attention. However, do not direct the air horn at any dog or individual. Point the air horn up and away from anyone's ears, and press the trigger button.
- Ensuring all dogs are on leashes and keeping them separated by supervision of both pets and pet owners greatly reduces the risks of dogfights.

Handling Small Dogs

Smaller dogs pose different restraint challenges. Their size can make them both easier and more difficult to restrain. Sometimes it is more difficult to keep a smaller dog still while restraining. Their small size allows them more wiggle room. Sometimes it is much harder to keep smaller legs and muzzles in grasp.

- Keep in mind that just because the dog is smaller, it does not mean that it is less dangerous. Small dogs can inflict very damaging bite wounds when frightened or provoked.
- When picking up a small dog, one hand should be under the chest for support with the front legs between fingers. The other hand should be used to support the rear end. Restraining the front legs between the fingers prevents the employee from being scratched.
- Utilizing a towel for restraint and lifting can be effective with the smaller dog. Utilize a towel to completely wrap up the dog "burrito" style. By swaddling them in a towel, they are unable to effectively wiggle out of grasp or scratch.
- Slowly lift the pet, keeping your face tipped up, away from the dog's muzzle.
- Restrain the dog by wrapping one arm under the front of the chest, snuggling up to the neck, and keeping your face away from the pet's muzzle.

Lifting Large Dogs

- Never attempt to lift a large dog without assistance.
- Never crouch by a dog unless you know it's safe. Stoop from the waist.
- When it is safe to proceed, bend knees and keep your face away from the dog's muzzle.
- When lifting a large dog, place one arm under its chest and in front of its forelegs, wrapping the top of the arm around the neck to keep the head from turning and therefore allowing a bite to occur.

- Place your second arm under the chest behind the front legs.
- Have the second employee place an arm under the abdomen in front of the hind legs.
- When lifting a large dog that has been incapacitated or under anesthesia:
- Two employees should transport the patient. Support the pet in the same manner as listed above except, they should be approaching the pet along its back, therefore supporting the spine and keeping the pet from flopping about.
- Push, do not pull a gurney or rolling stretcher, when transporting a large dog.

Walking Dogs

- Dogs should be on a leash at all times - NO EXCEPTIONS.
- Two leashes should be used for all dogs.
- Walk dogs in a secure and enclosed area if possible.
- Dogs should always go out one at a time even if they are from the same household.
- Never allow unrelated dogs to get within touching distance of each other. This applies when walking by, walking in or out of the facility, or placing them out in the kennel to clean cages, etc.

Handling and Managing Aggressive Dogs

Aggressive dogs are most likely to bite. All aggressive dogs have one body language characteristic in common: the “frozen” or “statue” posture. Dogs may attack without any other warning than the “frozen” stance.

- Using proper restraint techniques when handling aggressive dogs is paramount. Failure to do so could result in injury to the doctor, employee, patient and client.
- One arm cradles the neck and another arm under the belly or otherwise stabilizing the pet. Hold the pet firmly and securely in this position.
- This allows the pet to be worked on without moving or biting.
- Once you have secured an aggressive pet do not let go until all procedures are completed.
- When releasing the dog from restraint, do not do so until all parties in the area have been notified.
- Do not let your guard down; this is a time when many dogs take the opportunity to bite.

Removal From Cage

- Usually, the most difficult task when dealing with aggressive dogs is to remove them from the cage or kennel.
- Approach the dog in a calm manner; do not stare directly at them.
- Create a large loop in a slip leash; casually slip the lead over their head. Sometimes this can be accomplished without having to open the cage or kennel door all the way. Crack the door open, if the dog approaches the door, slip the large loop over the head through the opening in the door.
- If the dog is backed into a corner or behaving in an outwardly aggressive manner, do not put yourself at risk.

- Utilize a rabies pole or other device that extends your reach. Place a slip leash at the end of the device to slip over the dog's head.
- Using a towel over the head of smaller dogs first, before putting on a leash works well.
- After retrieving an aggressive dog, you may wish to leave the leash on the pet. Be sure to tie the leash securely to the outside of the cage. When using this technique, the pet must be monitored closely to be sure it does not choke itself.

Use of muzzle or other protective equipment

- Never hesitate to utilize a muzzle.
- If using a muzzle from the owner carefully examine it for defects. It is common for owners to put the muzzle on improperly and therefore may be dangerous.
- If impossible to muzzle, pull a leash through a door jamb or cage door, snugging the dog tightly against it. Quickly perform procedure and release.

Dominant Aggressive Dogs

- Guard type dogs usually fall under this category.
- Dominant/Aggressive dogs try to make themselves look as large as possible.
- Dominant/Aggressive dogs will present themselves at the front of the kennel or cage; hair erect, tail and ears up.
- They will often warn by bearing teeth (curled lips), growling and or barking.
- Not all dogs that wag their tails can be considered "safe", especially if in a different environment or around a stranger. Never take the sign of a wagging tail as a guarantee.
- Chow Chows are well known for vigorously wagging their tails before lunging and biting.

Fear Based Aggression in Dogs

The dog that exhibits fear-based aggression is just as dangerous as a dominant aggressive dog.

- The fear-based dog will try to bite because he thinks his life is in danger. These patients will often not give a warning before striking except to show the frozen posture.
- Encouraging the owner to bring the fearful aggressive and non-aggressive dog in for short visits and treats (without an examination) will greatly reduce fear and build confidence in the dog, and can often reduce the incidence of biting during future visits.
- Identify fear-based aggression by observing the following body postures
- Backed into a corner
- Tail and ears down
- Crouched over in order to appear as small as possible
- The fear-based dog may bear its teeth and growl as well

Non-Aggressive Fearful Dogs

Identify fearful non-aggressive dogs by observing the following body language these dogs may easily bite when subjected to pain:

- Submissive urination

- Rolling over and showing belly
- Quivering

Fear Free Certified

If your Clinic is fear free certified by AAHA, employees are exempt from using the above techniques that may be characterized as forceful restraint per AAHA fear-free certified standards. All other applicable GREEN DOG safety precautions should be taken.

CLINIC OPERATIONS

Compressed Gas Safety

Veterinary Clinics have a wide variety of compressed gases in their inventory. These items are stored in gas cylinders and range from oxygen cylinders to aerosol cans. The following safety procedures should always be followed when handling these compressed gases:

- Store cylinders only above ground in a definite, designated, labeled storage area.
- Gas cylinders containing oxygen, nitrous oxide, or nitrogen (or other substances) should be stored in an upright position. Always keep the steel protective cap screwed on. This step reduces the chance that a blow to the valve will allow gas to escape.
- Group cylinders by types of gas.
- Gas cylinders must be securely anchored so they may not tip over or roll. Secure cylinders with chains or cables to keep them from falling over. (If a tank falls over, causing the valve to break off, then you have a 100lb. "balloon" missile flying around trying to kill people!!!)
- Store full and empty cylinders apart. Gas cylinders must be properly labeled showing contents and label empty cylinders and those meant for return to the supplier.
- Store gases so that old stock is removed and used first.
- Store compressed gas containers in dry, well-ventilated areas away from exits and stairways. If stored out of doors, containers should be off the ground and out of extremely hot or cold environments.
- Do not store compressed gas containers in high pedestrian and vehicle traffic areas. (At least 25 feet from congested areas such as offices, lunchrooms, or places where people congregate.)
- Store oxygen cylinders at least 20 feet from flammables or combustibles or separate them by a 5-foot, fire-resistant barrier.
- Keep oil and grease away from oxygen cylinders, valves, and hoses. Clean cylinders to remove oil or grease (including oil from hands).
- If your hands, gloves, or clothing are oily, do not handle oxygen cylinders.
- Make sure fire extinguishers near the storage area are appropriate for gases stored there.
- Post signs stating the name(s) of gas present (e.g. "OXYGEN") and "NO SMOKING" and "NO OPEN FLAMES" signs where gases are stored.
- When transporting cylinders, do not roll, bang or drop them, use a hand truck or dolly.
- All cylinders when not in use must have a collar in place over the valve.
- Never compress aerosol cans when disposing.

- Do not puncture an aerosol can.
- Dispose of empty aerosol cans in the trash.
- Do not dispose of ½ used aerosol cans, take them to a community hazardous waste collection site.
- Cylinders containing Nitrous Oxide should be closely monitored as they have the potential to be abused.

Mercury

Humans use mercury in a variety of manufacturing processes and products such as thermometers and fluorescent bulbs (CFLs). If you improperly dispose of products with mercury in them, they may break and release mercury vapors which are harmful to human and ecological health.

If a thermometer breaks:

1. Put on rubber, nitrile or latex gloves.
2. If there are any broken pieces of glass or sharp objects, pick them up with care. Place all broken objects on a paper towel. Fold the paper towel and place in a zip lock bag. Secure the bag and label it as directed by your local health or fire department.
3. Locate visible mercury beads. Use a squeegee or cardboard to gather mercury beads. Use slow sweeping motions to keep mercury from becoming uncontrollable. Take a flashlight, hold it at a low angle close to the floor in a darkened room and look for additional glistening beads of mercury that may be sticking to the surface or in small cracked areas of the surface. Note: Mercury can move surprising distances on hard-flat surfaces, so be sure to inspect the entire room when "searching."
4. Use the eyedropper to collect or draw up the mercury beads. Slowly and carefully squeeze mercury onto a damp paper towel. Place the paper towel in a zip lock bag and secure it. Make sure to label the bag as directed by your health or fire department.
5. After you remove larger beads, put shaving cream on top of a small paint brush and gently "dot" the affected area to pick up smaller hard-to-see beads. Alternatively, use duct tape to collect smaller hard-to-see beads. Place the paint brush or duct tape in a zip lock bag and secure it. Make sure to label the bag as directed by your local health or fire department.
6. Place all materials used with the cleanup, including gloves, in a trash bag. Place all mercury beads and objects into the trash bag. Secure trash bag and label it as directed by your local health or fire department.
7. Contact your local health department, municipal waste authority or your local fire department for proper disposal in accordance with local, state and federal laws.
8. Remember to keep the area well ventilated to the outside (i.e., windows open and fans in exterior windows running) for at least 24 hours after your successful cleanup.

If a Compact Fluorescent Light (CFL) breaks:

1. Before clean-up, air out the room
2. Have people and pets leave the room, and don't let anyone walk through the breakage area on their way out.

3. Open a window and leave the room for 15 minutes or more.
4. Shut off the central forced-air heating/air conditioning system, if you have one.
5. For hard surfaces
 - a. Carefully scoop up glass pieces and powder using stiff paper or cardboard and place them in a glass jar with a metal lid (such as a canning jar) or in a sealed plastic bag.
 - b. Use sticky tape, such as duct tape, to pick up any remaining small glass fragments and powder.
 - c. Wipe the area clean with damp paper towels or disposable wet wipes. Place towels in the glass jar or plastic bag.
 - d. Do not use a vacuum or broom to clean up the broken bulb on hard surfaces.
6. For carpeting or rug
 - a. Carefully pick up glass fragments and place them in a glass jar with a metal lid (such as a canning jar) or in a sealed plastic bag.
 - b. Use sticky tape, such as duct tape, to pick up any remaining small glass fragments and powder.
 - c. If vacuuming is needed after all visible materials are removed, vacuum the area where the bulb was broken.
 - d. Remove the vacuum bag (or empty and wipe the canister), and put the bag or vacuum debris in a sealed plastic bag.
 - e. Disposal of Clean-up Materials
7. Immediately place all clean-up materials outdoors in a trash container or protected area for the next normal trash pickup.
8. Wash your hands after disposing of the jars or plastic bags containing cleanup materials.
9. Check with your local or state government about disposal requirements in your specific area. Some states do not allow such trash disposal. Instead, they require that broken and unbroken mercury-containing bulbs be taken to a local recycling center.

What Never to Do with a Mercury Spill

- Do not allow people in the area, or to walk through the area
- Do not vacuum up the mercury, this can aerosolize it. Instead use a card such as a cage card to “round-up” the spilled mercury droplets into a plastic bag
- Never use a broom to clean up mercury. It will break the mercury into smaller droplets and spread them.
- Do not attempt to clean up without the use of gloves
- Do not dispose of mercury without first contacting your local health department for information on mercury disposal.
- Do not dispose of mercury in the regular trash and never pour mercury down a drain.
- Never wash clothing or other items that have come in direct contact with mercury in a washing machine. Clothing, including shoes that have come into direct contact with mercury should be discarded.

Autoclave

The autoclave uses pressurized steam to kill microorganisms that may be hiding in surgical and veterinary tools. Employees must be aware of the various hazards associated with autoclaves and therefore, must use caution when operating it.

- Burn hazards- Steam from the autoclave is extremely hot and can cause serious burns. Caution must be taken when opening the door to avoid contacting the escaping steam and heat. Caution must also be taken when loading and unloading, as contents inside will be extremely hot.
- Explosion hazards- Explosions are rare occurrences, but can happen when the door seal malfunctions or when the autoclave is loaded improperly. Pressure and heat in the chamber will escape rapidly with a potential for serious injury and property damage.
- Biological hazards- An overloaded autoclave will prevent items from sterilizing completely and may cause cross-contamination or transmission of infection to other patients. Use biological indicators or sterile strips to ensure items have been properly decontaminated.
- Heavy lifting hazards- Trays that require lifting may cause strain if proper lifting techniques are not used. Always use proper lifting techniques when moving or carrying items.

Safety Guidelines:

- Hazardous chemicals should NEVER be placed in an autoclave. Autoclaving oxidizing (e.g., bleach) or flammable liquids can cause an explosion.
- Once the cycle is complete, wait until the pressure and temperature drop before unlatching the door. Keep your face away from the door.
- Never open the autoclave while it is in the process of sterilization.
- Do NOT autoclave radioactive materials or items containing corrosives (acids, bases, phenol).
- If unsure if an item can be sterilized in an autoclave, check with the product's manufacturer guidelines.
- Do not stack or store combustible material next to an autoclave (cardboard, plastic, volatile or flammable liquids).
- Place autoclave in a low traffic area to prevent injury or damage in case of an explosion.
- Do not touch the autoclave while it's running. Even though most modern autoclaves have a built in shield to protect from thermal burns, the machine's high temperatures can create severe burns if it is touched while running or still hot.
- Ensure to inspect autoclave regularly and perform maintenance on a yearly basis or according to manufacturer recommendations. Dates of inspection and replacement must be recorded in the Clinic Safety Maintenance Binder.
- If possible, avoid removing items from the autoclave immediately after the cycle. Wait for the items to cool.
- Steam may cause rusting of carbon steel items, so check items to ensure items are in good condition.

- If the autoclave is not working properly, immediately tag it so that it is not used and have it serviced.

Surgical Equipment

Always take care when handling surgical equipment. Many pieces of surgical equipment and instruments have sharp edges that can puncture a careless employee.

- Take special care when working with surgical pins.
- Do not carry sharp pieces of surgical equipment with the sharp tip facing your body.
- Identify and properly eliminate sharps when cleaning a surgical instrument table after use.

Zoonoses

Veterinary practices are unique environments that bring humans into close contact with many species of animals and some of these animals may transmit diseases to people. These diseases are called Zoonotic Diseases. The spectrum of diseases, their severity and ease of transmission will vary from practice to practice and between geographic locations. Veterinary employees have an increased potential of being exposed to Zoonoses because of the nature of the type of work they are doing. However, to ensure employee safety, GREEN DOG has implemented an Infection Control Plan. By following universal precautions, proper safety procedures, and wearing personal protective equipment, employees can safely do their job without falling ill.

Direct Contact Zoonotic Diseases

Transmission of zoonotic diseases may occur when a human is exposed to an animal bite; scratches from cats; contact with lesions on animals; contact with an infectious animal's blood, body fluids, feces, etc.; infectious needlestick or sharp injury; cleaning cages; aerosol transmission; vector-borne transmission from animals bringing in fleas, mosquitoes, or ticks; etc. Examples of zoonotic diseases are:

- Ringworm
- Giardia
- Leptospirosis
- Rabies
- Anthrax
- Methicillin-resistant Staphylococcus aureus (MRSA) infection
- Salmonella and E. coli
- Influenza viruses
- Toxoplasmosis
- Plague
- West Nile Virus

Preventing Zoonotic Contamination

The most effective defense against disease transmission is practicing good hygiene. Good hygiene begins with thorough hand washing. Wash hands before and after each patient encounter

and after contact with feces, blood, body fluids, secretions, excretions, exudates, or articles contaminated by these substances. Wash hands before eating, drinking, or smoking; after using the toilet; after cleaning animal cages or animal-care areas; and whenever hands are visibly soiled. Alcohol-based rubs may be used if hands are not visibly soiled, but hand-washing with soap and running water is preferred. Keep fingernails short. Do not wear artificial nails or hand jewelry when handling animals. Keep hand-washing supplies stocked at all times.

Proper Hand Washing Procedures

(Recommended by CDC- The Centers for Disease Control and Prevention)

1. When washing hands with soap and water:
2. Wet your hands with clean running water and apply soap. Use warm water if it is available.
3. Rub hands together to make a lather and scrub all surfaces.
4. Continue rubbing hands for 15-20 seconds. Need a timer? Imagine singing "Happy Birthday" twice through to a friend.
5. Rinse hands well under running water.
6. Dry your hands using a paper towel or air dryer. If possible, use your paper towel to turn off the faucet.
7. Always use soap and water if your hands are visibly dirty.
8. If soap and clean water are not available, use an alcohol-based hand rub to clean your hands.

Alcohol-based hand rubs significantly reduce the number of germs on skin and are fast acting.

1. When using an alcohol-based hand sanitizer:
2. Apply the product to the palm of one hand.
3. Rub hands together.
4. Rub the product over all surfaces of hands and fingers until hands are dry.

Personal Protective Equipment (PPE), such as gloves, eye goggles, face shields, etc. can greatly reduce the risk of zoonosis transmission by providing barrier protection. In addition to disposing of waste that is deemed zoonotic in the biohazardous waste bins.

Zoonotic Prevention Procedures

- Always wash your hands after and between handling patients.
- Always be sure to wash your hands before eating, drinking, or smoking.
- Keep human food items in "food only" designated areas.
- Food for human consumption should never be located in the lab area (this includes items such as coffee cups).
- Food for human consumption must be stored in a separate refrigerator. It must NOT be stored with vaccines or other biologicals, including lab samples.

- Always wear goggles, latex gloves and a mask or face shield when performing dentistry. Bacteria in the mouth of animals can be aerosolized which means that it can be easily inhaled by the technician or come in contact with the eyes causing infection.
- Be sure to wear gloves when handling animals with lesions or patches of hair missing.
- Always wash hands and work areas thoroughly.
- When cleaning or disinfecting work areas, always wear gloves.
- Avoid using spray bottles when disinfecting contaminated work area or cages. Spray bottles can aerosolize the bacteria or zoonotic disease and can easily be transmitted to personnel.
- All blood, tissues, body fluids from animals diagnosed with zoonotic diseases must be discarded as biohazardous waste.

Sharps Injuries

The most serious hazard from sharps in a veterinary medical environment is from the physical trauma (and possible bacterial infection) that is caused by a puncture or laceration.

To prevent accidents from punctures or lacerations:

- Always keep sharps, needles, scalpel blades and other sharp instruments capped or sheathed until ready for use.
- The needle or sharp should be placed in a red biohazard "sharps container" immediately after use.
- Do not attempt to recap the needle unless the physical danger from sticks or lacerations cannot be avoided by any other means. [Needles may ONLY be recapped by using the "one-handed" method: place the cap on a flat surface (like the table or counter) then, with one hand, "thread" the needle into the cap. The cap may then be firmly seated using both hands.]
- Needles should never be removed from the syringe, but the entire unit should be disposed of in the red biohazard sharps containers.
- When full, the sharps containers must be sealed and disposed of following the

Waste Management Plan.

- The cutting of needles prior to disposal increases the potential for aerosolization and collecting sharps in a smaller container and transferring them to a larger container for disposal places someone at an increased risk of exposure; neither practice is recommended.
- Never throw needles or sharps directly into regular trash containers, regardless of whether or not they are capped. Never open a used sharps container or stick your fingers into one for any reason.

Grooming & Bathing Safety Procedures

When bathing and grooming patients, employees may be exposed to pesticides and chemicals. Therefore it is important that employees familiarize themselves with the chemicals they are using

by referring to the MSDS. Pesticides and other chemicals can be found in shampoos, flea dips, medicated dips, and various pesticide sprays.

- Always read instructions on the label.
- Appropriate personal protective equipment (PPE) has been provided for employee use, it is mandatory that these items are used when bathing pets.
- Gloves, goggles and aprons must be used every time a patient is dipped.
- Non slip rubber mats must be used in the bathing area.
- Put out “wet areas” warning signs.
- Ensure electrical devices and/or extension cords are not exposed to moisture.
- Do not arrange extension cords so that they cause trips or falls. If it is necessary to run an extension cord a long way, secure it with duct tape to keep the cord out of the path of other employees and patients.
- Excess water in the area should be mopped up immediately.
- Wear ear plugs when working in high decibel areas (dryers, barking dogs).
- Always clean up after yourself. Keep the work area clean and organized.
- Thoroughly wash hands and arms after bathing each pet.
- Never lift a heavy dog without assistance.

COMMON SENSE

No manual, no matter how detailed can cover all potentially hazardous situations. It is imperative that all employees assess each situation and make some decision about the risk that might occur to them and their co-workers. If a situation arises where you do not feel adequately trained to assure everyone’s safety, do not proceed. Contact your supervisor and get the training and help you need. In many situations common sense will be the determining factor between the safe and enjoyable performance of one’s job and a serious problem.

A FINAL NOTE

GREEN DOG works for you – our Clinic Team Members. Our job is to help you understand and implement the Clinic Safety Program. We encourage questions and will help in any way possible.

ACKNOWLEDGEMENT OF RECEIPT OF EMPLOYEE SAFETY HANDBOOK AND PROMISE TO COMPLY

I, the undersigned, hereby acknowledge receipt of GREEN DOG’s Employee Safety Handbook. I acknowledge and understand that it is my responsibility to read and comply with this Handbook and to ask my supervisor or any member of management any questions that I may have about the

Company's safety policies, rules or practices. I understand that GREEN DOG Inc. is an "at-will" employer. Therefore, nothing stated within this Acknowledgement or Handbook is intended to alter, modify or eliminate the at-will status of employment with GREEN DOG Inc. Either I or GREEN DOG Inc. may terminate the employment relationship at any time with or without cause and with or without prior notice.